Connect the Dots
A map of co-occurring conditions

Neurodivergent Wellbeing Scale

Two questions are better than one

"How are you out of 10?" assumes we can identify how we're feeling and convert it to a number. For many of us, the energy runs out long before it shows on the outside. This scale asks two things: **how we're doing** and **how we feel about it**.

How to use: Find the description that matches right now in each column and point to it. If something is hard to say out loud, point to the safety box below the scale.
The *italic notes* are for clinicians.

HOW I'M DOING

I'm doing well. I have energy to spare. I'm engaged with things I enjoy. My brain feels clear and things feel manageable.
May not recognise this state if it's unfamiliar. May not last, and knowing that can be its own stress.

I'm getting by. I'm getting through the day but my energy is stretched. I'm keeping up but I'm aware of the cost.
"Okay" may already be lower than a clinician assumes. The mask is holding but the battery is draining.

I'm struggling. My energy is running out. I'm dropping non-essential tasks. Social interaction costs too much. I need recovery time after everything.
May look like avoidance or laziness. It's depletion. Executive function decline may be the only visible sign.

I'm barely coping. I don't have the energy for basic things. Care tasks are being missed. I can't plan or make decisions.
May present as "quiet" rather than distressed. Absence of distress signals doesn't mean absence of distress.

I have nothing left. I've disconnected. I may not be able to tell you what's wrong.
Shutdown can look like calm. May still answer questions on autopilot. The mask is the last thing to go.

HOW I FEEL ABOUT IT

I feel good. This feels right. I'm where I want to be, or I'm getting there.
At lower functional levels, check whether this is genuine acceptance or baseline normalisation.

I'm not sure. Something feels off but I can't name it.
This is the alexithymia signature. Not being able to answer IS the answer.

I want this to change. I'm not okay with where I am but I don't know how to shift it.
May have been here long enough that wanting change feels hopeless rather than motivating.

It's too much. I'm overwhelmed and I can't manage how I feel.
May present as irritability, rigidity, withdrawal, or increased need for routine, not as distress.

I've gone past it. I'm not engaging with how I feel anymore.
This can look like calm or indifference. It's not; it's overload. The feelings haven't gone, they've disconnected.

Some things are hard to say out loud:

I also need to tell you...

- I'm having thoughts of hurting myself
- I'm having thoughts about not wanting to exist
- I'm having thoughts about ending my life
- I'm having thoughts about hurting someone else

These thoughts can happen at any level above, including "doing well."
Pointing to this box is not dramatic. It is communicating something important.

Lifeline 13 11 14 | Beyond Blue 1300 22 4636 | Kids Helpline 1800 55 1800 | Emergency 000

For clinicians - how to read the combination:

	GOOD	UNSURE	CHANGE	OVERWHELM	PAST IT
Doing well	Stable	Explore	Explore	Support	Assess ⚑
Getting by	Stable	Explore	Support	Escalate	Escalate
Struggling	Explore ⚑	Support	Support	Escalate	Urgent
Barely coping	Assess ⚑	Escalate	Escalate	Urgent	Crisis
Nothing left	Assess ⚑	Urgent	Urgent	Crisis	Crisis

⚑ Reported feeling doesn't match reported state. May indicate normalisation, dissociation, or masking.

This is a communication tool, not a diagnostic instrument. It does not replace clinical assessment or risk assessment. If you are in crisis, contact a crisis service or go to your nearest emergency department.

CC BY-NC-SA 4.0 | v1.0 · March 2026